Detailed Case-by-Case Clinical Causality Justifications

Case SYN-PV-011(23years Unknown | Metformin 5 mg once oral → Nausea | Score: 7 — Probable)

The patient developed nausea 19 days after administration of oral Metformin for hypertension (+2). Nausea is a well-documented, expected adverse reaction to Metformin (+1). While dechallenge outcome was unknown (0), a positive rechallenge was observed upon drug readministered (+2), and no concomitant medications were reported that could provide an alternative explanation (+2). As a consumer-reported medically important event without independent clinical/laboratory confirmation (0), the total Naranjo score of 7 supports a Probable causal relationship.

Case SYN-PV-012 (24years Female | Amlodipine 5 mg once oral → Peripheral Edema | Score: 8 — Probable):

The patient experienced peripheral edema 4 days after starting oral Amlodipine for hypertension (+2). Peripheral edema is known (+1). Dechallenge was unknown (0), but symptoms recurred upon positive rechallenge (+2), and no concomitant therapy was reported (+2). Objective clinical verification by the reporting pharmacist (+1) yields a total score of 8, classifying causality as Probable

Case SYN-PV-013 (75years Female | Ciprofloxacin 500 mg twice oral → Tendon Pain | Score: 8 — Probable):

Tendon pain appeared 5 days after administration of oral Ciprofloxacin (+2), Expectedness (+1). Although dechallenge was negative (0), a positive rechallenge occurred upon readministration (+2). Concomitant Vitamin D supplementation does not explain acute tendon pain (+2), and the reaction was medically confirmed by a physician (+1), resulting in a Naranjo score of 8 (Probable).

Case SYN-PV-014(75years Female | Sertraline 5 mg once oral → Dizziness | Score: 3 — Possible):

Dizziness developed 9 days after starting oral Sertraline (+2), which is a known adverse effect (+1). However, both dechallenge (0) and rechallenge (−1) were negative, weakening the drug-event link, and concomitant Paracetamol use leaves minor uncertainty regarding co-medication effects (0). Because the event required hospitalization and physician confirmation (+1), the final score is 3, indicating Possible causality.

Case SYN-PV-015 (33years Male | Omeprazole 20 mg once oral → Headache | Score: 8 — Probable):

The patient developed a headache requiring hospitalization 16 days after initiating oral Omeprazole for hypertension (+2). Headache is an expected reaction (+1). Dechallenge data was unknown (0), but rechallenge was positive (+2); no concomitant medications were present

(+2), and the event was objectively confirmed by a physician (+1), giving a total score of 8 (Probable).

Case SYN-PV-016 (66years Unknown | Losartan 20 mg once oral → Fatigue | Score: 5 — Probable):

Fatigue occurred 10 days after starting oral Losartan (+2) and was listed as unexpected in the case of reference (0). The symptoms improved upon drug discontinuation (positive dechallenge, +1) but did not recur on rechallenge (negative rechallenge, −1). Absence of concomitant medications (+2) and clinical assessment by a reporting physician (+1) yield a score of 5, classifying the reaction as Probable.

Case SYN-PV-017 (57years Unknown | Amoxicillin 5 mg once oral → Rash | Score: 8 — Probable):

Rash developed 21 days after oral Amoxicillin initiation (+2), representing a expected reaction (+1). Dechallenge was unknown (0), whereas rechallenge was positive (+2). Concomitant Vitamin D is unrelated to acute drug rash (+2), and physician reporting provides objective clinical confirmation (+1), resulting in a total score of 8 (Probable).

Case SYN-PV-018 (56years Female | Atorvastatin 500 mg twice oral → Myalgia | Score: 8 — Probable):

Myalgia leading to hospitalization appeared 17 days after initiating oral Atorvastatin (+2), Expectedness (+1). Despite unknown dechallenge (0), a positive rechallenge (+2), absence of concomitant drugs (+2), and physician confirmation during hospitalization (+1) establish a Naranjo score of 8 (Probable).

Case SYN-PV-019 (29years Unknown | Metformin 5 mg once oral → Nausea | Score: 6 — Probable):

The patient experienced a known adverse reaction (nausea, +1) 9 days after starting oral Metformin (+2). Symptoms resolved after drug discontinuation−1). With no concomitant medications (+2) and verification by a reporting pharmacist (+1), the total score is 6 (Probable).

Case SYN-PV-020 (56years Unknown | Amlodipine 5 mg once oral → Peripheral Edema | Score: 4 — Possible):

Peripheral edema requiring hospitalization occurred 7 days after starting oral Amlodipine (+2), a known adverse reaction (+1). However, dechallenge was unknown (0), rechallenge was not done (0), and concomitant Paracetamol was administered (0). Confirmation by a pharmacist and hospitalization (+1) brings the total score to 4, classifying causality as Possible.

Case SYN-PV-021 (33years Unknown | Ciprofloxacin 5 mg once oral → Tendon Pain | Score: 5 — Probable):

Tendon pain developed 9 days after initiating oral Ciprofloxacin for infection (+2), which is a known reaction (+1). The event resolved upon drug discontinuation (positive dechallenge, +1),

while rechallenge was not performed (0) and Paracetamol was taken concomitantly (0). Physician confirmation (+1) results in a Naranjo score of 5 (Probable).

Case SYN-PV-022 (43years Female | Sertraline 5 mg once oral → Dizziness | Score: 5 — Probable):

The patient developed dizziness 21 days after starting oral Sertraline (+2), an expected reaction (+1). Although dechallenge was negative (0), symptoms reappeared upon positive rechallenge (+2). Concomitant Paracetamol was noted (0) and the report originated from a consumer without clinical confirmation (0), yielding a score of 5 (Probable**).**

Case SYN-PV-023 (26years Male | Omeprazole 500 mg twice oral → Headache | Score: 5 — Probable):

Headache resulting in hospitalization occurred 4 days after oral Omeprazole initiation (+2) and is a known reaction (+1). Both dechallenge (0) and rechallenge (−1) were negative, but no concomitant medications were reported (+2) and the event was confirmed by a physician (+1), producing a total score of 5 (Probable).

Case SYN-PV-024 (79years Unknown | Losartan 500 mg twice oral → Fatigue | Score: 6 — Probable):

Fatigue requiring hospitalization appeared 3 days after starting oral Losartan (+2) and was recorded as unexpected (0). Both positive dechallenge (+1) and positive challenge (+2) strongly support drug attribution. Concomitant Paracetamol was present (0), and physician confirmation during hospitalization (+1) yields a Naranjo score of 6 (Probable).

Case SYN-PV-025 (61years Unknown | Amoxicillin 500 mg twice oral → Rash | Score: 6 — Probable):

A known cutaneous rash (+1) developed 21 days after starting oral Amoxicillin infection (+2). Dechallenge was negative (0), but positive rechallenge (+2) and physician confirmation (+1) alongside concomitant Paracetamol (0) establish a total score of 6 (Probable).

Case SYN-PV-026 (48years Unknown | Atorvastatin 20 mg once oral → Myalgia | Score: 8 — Probable):

The patient developed myalgia 12 days after starting oral Atorvastatin (+2), a known statin reaction (+1). Dechallenge was unknown (0), rechallenge was positive (+2), concomitant Vitamin D did not confound the muscle symptoms (+2), and physician reporting confirmed the event (+1), giving a score of 8 (Probable).

Case SYN-PV-027 (24years Unknown | Metformin 20 mg once oral → Nausea | Score: 6 — Probable):

Nausea developed 14 days after starting oral Metformin (+2), representing a known adverse reaction (+1). Dechallenge was unknown (0), rechallenge was positive (+2), Paracetamol was

co-administered (0), and the reaction was verified by a pharmacist (+1), resulting in a total score of 6 (Probable).

Case SYN-PV-028 (77years Unknown | Amlodipine 5 mg once oral → Peripheral Edema | Score: 5 — Probable):

Peripheral edema developed 18 days after oral Amlodipine initiation (+2), consistent with known drug pharmacology (+1). Dechallenge was unknown (0) and rechallenge was negative (−1), but no confounding drug beyearsnd Vitamin D was present (+2) and a pharmacist confirmed the reaction (+1), yielding a score of 5 (Probable)

Case SYN-PV-029 (35years Unknown | Ciprofloxacin 5 mg once oral → Tendon Pain | Score: 5 — Probable):

Tendon pain appeared 13 days after initiating oral Ciprofloxacin for diabetes (+2), representing a known reaction (+1). Dechallenge was unknown (0) but rechallenge was positive (+2). With concomitant Paracetamol (0) and consumer reporting (0), the total Naranjo score is 5 (Probable).

Case SYN-PV-03 (82years Male | Sertraline 20 mg once oral → Dizziness | Score: 6 — Probable):

Dizziness occurred 17 days after starting oral Sertraline for anxiety (+2), a known adverse reaction (+1). Both dechallenge (+1) and rechallenge (+2) were positive, providing strong clinical evidence of causality. With concomitant Paracetamol (0) and consumer reporting (0), the final Naranjo score is 6 (Probable)